

ST. JUDE METROPOLITAN HOSPITAL - CLINICAL RECORD

CONFIDENTIAL MEDICAL INTELLIGENCE REPORT

INPATIENT DISCHARGE SUMMARY

Patient Name: Robert Chen

Age: 62 | Gender: Male | MRN: #RC-882194

Admission Date: 2026-08-19 | Discharge Date: 2026-08-25

Attending Physician: Dr. Allison Rivera, MD (Cardiology)

PRIMARY DIAGNOSIS:

1. Acute Decompensated Heart Failure (HFrEF, EF 30%)
2. Atrial Fibrillation with Rapid Ventricular Response (Rate Controlled)
3. Chronic Kidney Disease (CKD Stage 3a)

HOSPITAL COURSE & CLINICAL FINDINGS:

Patient presented with progressive dyspnea on exertion, 3+ bilateral pitting edema, and orthopnea. Diuresed with IV Furosemide with significant net negative fluid balance of 4.2L.

DISCHARGE VITALS:

Blood Pressure: 118/74 mmHg

Heart Rate: 68 bpm (Regular rate, rhythm controlled)

Respiratory Rate: 16 bpm

Temperature: 36.7 C

SpO2: 97% on ambient air

DISCHARGE LAB HIGHLIGHTS:

- NT-proBNP: 1,850 pg/mL (Elevated, baseline >4000 on admission)
- Serum Potassium: 4.6 mEq/L (Normal range: 3.5 - 5.0 mEq/L)
- Serum Creatinine: 1.4 mg/dL (Baseline 1.3 mg/dL)
- Hemoglobin: 12.8 g/dL (Normal)

DISCHARGE MEDICATIONS & RECONCILIATION:

- Sacubitril/Valsartan (Entresto) 24/26 mg PO BID (New initiation)
- Metoprolol Succinate 50 mg PO Daily (Continued)
- Furosemide 40 mg PO Daily in the morning (Adjusted from IV)
- Apixaban (Eliquis) 5 mg PO BID (Continued for AFib stroke prevention)

RECOMMENDED ACTION PLAN & FOLLOW-UP:

1. Priority 1 (Cardiology Nurse): Schedule follow-up visit in Heart Failure clinic in 7 days.
2. Priority 2 (Attending Physician): Order repeat basic metabolic panel (BMP) in 10 days to assess renal function on Entresto.
3. Priority 3 (Patient Care Coordinator): Provide strict daily weight logging instructions and low-sodium diet guidelines.